

2nd week Monday case scenario

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(Hypertensive disorder in pregnancy)

Case 32

A 16-year-old primigravida presents to emergency unit at 35 weeks gestation. Her blood pressure is 170 / 110 and she has 4+ proteinuria on a clean catch specimen of urine. She has significant swelling of her face and extremities. She denies having contractions. Her cervix is closed and unaffected. The baby is cephalic by bedside ultrasonography. She says the baby movements have decreased in the past 24 hours.

32.1 What is the next step in the management of this patient?

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32.2 What is the best position to measure her blood pressure and why?

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32.3 Clarify the physiological effect of pregnancy on blood pressure?

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32.4 Dose the swelling of extremities significant during pregnancy?

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32.5 Is there a role of antihypertensive in the management of this case and what is the best drug of choice, why?

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32.6 Is there an indication for termination of pregnancy in this gestational age?

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32.7 Which is the best method for termination?

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Her liver function test: Alkaline
phosphatase=40KAU,SGOT=60U/L,SGPT=22U/Land
Albumin=1.3g/dl,TSB2.8mg/dl.

32.8 What further investigation you should send and what is the
complication might be developed?

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The patient 24 after delivery was develop dyspnea, SOB, tachycardia,
and by auscultation of the chest there is bilateral basal crepitation.

32.9 What complication was thought to be developed in this patient?

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